# Clinical Escalation and Scope-of-Practice Standard

MERIDIAN HEALTH PARTNERS
OPERATIONS POLICY MANUAL
Policy ID: MHP-POL-2024-041
Subject: Non-Clinical Communications and Patient Inquiry Routing Standard
Effective Date: November 1, 2024
Applies To: All non-clinical staff, receptionists, call center agents, scheduling personnel, automated interactive voice response (IVR) systems, and patient portal automated messaging bots across all 14 Meridian Health Partners sites.
Approved By: Dr. Aris Thorne, Chief Medical Officer; Brenda Vance, Director of Clinical Operations.

## 1. PURPOSE AND SCOPE

1.1 This policy defines the strict boundaries of communication for all non-clinical personnel and automated communication systems within Meridian Health Partners. To ensure patient safety, regulatory compliance, and mitigate clinical liability, non-clinical staff and automated platforms are legally and operationally prohibited from providing clinical interpretations, medical advice, or treatment guidance.

1.2 This standard applies unconditionally across all communication channels, including in-person front desk interactions, inbound and outbound telephone calls, the patient portal messaging interface, automated SMS reminders, and IVR routing prompts.

## 2. DEFINITIONS

2.1 Non-Clinical Staff: Any employee, contractor, or vendor representative at Meridian Health Partners who does not hold an active, unencumbered license as a Medical Doctor (MD), Doctor of Osteopathic Medicine (DO), Nurse Practitioner (NP), Physician Assistant (PA), Registered Nurse (RN), or Licensed Practical Nurse (LPN).

2.2 Automated Systems: Any software-driven communication tool deployed by Meridian Health Partners, including chatbots, IVR trees, automated email responders, and SMS triage filters.

2.3 Clinical Inquiry: Any patient communication that touches upon diagnosis, treatment, pharmacology, physiology, pathology, or personal health management.

## 3. MANDATORY ROUTING CATEGORIES

3.1 Non-clinical staff and automated systems are strictly prohibited from answering, interpreting, or speculating on any patient inquiry falling into the categories listed below. Every such inquiry must be immediately routed to a licensed clinician in accordance with Section 4.

3.2 Category 1: Symptom Interpretation. Any question asking staff to evaluate, diagnose, explain, or assess physical or psychological symptoms (e.g., "Is this rash normal?", "Could this chest tightness be anxiety?", "How long should a cough last?").

3.3 Category 2: Medication Questions. Any question concerning the purpose, mechanism, side effects, interactions, or appropriateness of a prescribed or over-the-counter drug (e.g., "What is this pill for?", "Can I take ibuprofen with my prescription?", "Will this make me dizzy?").

3.4 Category 3: Dosage Inquiries. Any question regarding how much of a medication to take, frequency of administration, titration schedules, or missed doses (e.g., "Should I take two pills if I missed the morning dose?", "Is 50mg the right amount for my weight?").

3.5 Category 4: Test-Result Meaning. Any question asking for the interpretation, clinical significance, or contextual meaning of laboratory values, imaging reports, pathology slides, or diagnostic traces (e.g., "What does a high white blood cell count mean?", "Is my MRI clear?", "Are these numbers normal?").

3.6 Category 5: "Should I" Inquiries. Any question phrased with the words "should I" or requesting a behavioral, clinical, or therapeutic recommendation (e.g., "Should I stop taking my medication?", "Should I go to the emergency room?", "Should I cancel my appointment if I have a fever?", "Should I put ice or heat on this?").

## 4. MANDATORY DEFLECTION PROTOCOL AND SCRIPTS

4.1 When a patient presents any inquiry listed in Section 3, the non-clinical staff member or automated system must immediately cease attempting to answer the question, apply the deflection script verbatim, and execute the routing procedure within the electronic health record (EHR) within 90 seconds.

4.2 Verbal Deflection Script (Phone and In-Person):
> *"I understand you are looking for guidance on that, but as a non-clinical team member, I am not permitted to provide medical advice, interpret clinical information, or answer questions about symptoms, medications, or test results. Let me route your message directly to your care team’s clinical triage queue so a licensed nurse or provider can review it and call you back."*

4.3 Written/Digital Deflection Script (Patient Portal and Automated Chatbots):
> *"I am an automated assistant / non-clinical staff member and cannot provide medical advice or answer clinical questions. Your message has been flagged and forwarded to your provider’s clinical team for review. A clinical staff member will respond via the portal or phone within the standard triage window."*

4.4 Routing Requirements:
* Phone/In-Person: Staff must generate an urgent clinical encounter note in the Meridian EHR (Epic/Cerner equivalent) and route it to the specific provider's nursing pool with a tag matching the inquiry category.
* Automated Systems: The bot or IVR must immediately terminate the automated response loop, assign a high-priority flag to the inbound message thread, and place it in the unread clinical triage pool for the patient's primary care team or specialty clinic.

## 5. ABSOLUTE ZERO-EXCEPTION RULE

5.1 The prohibitions and mandatory routing protocols outlined in this policy contain no exceptions whatsoever.

5.2 Repeat Callers: The rules apply identically to patients who call multiple times a day, frequent users of the clinic, and individuals who express frustration with the deflection script. Staff must repeat the exact deflection script without modification.

5.3 Healthcare Professionals: The rules apply without exception when a patient states, demonstrates, or provides documentation that they are a physician, nurse, pharmacist, or other licensed healthcare professional. Non-clinical staff and automated systems are strictly barred from engaging in peer-to-peer clinical dialogue or confirming clinical data to self-identified medical professionals. All such inquiries must be routed through the standard clinical queue.

5.4 Emergency Exceptions: If a patient describes an immediate, life-threatening emergency (e.g., active chest pain, stroke symptoms, severe hemorrhage, active suicidal ideation with intent), non-clinical staff and automated systems are exempt from Section 4.3 routing timelines only to the extent that they must immediately instruct the caller to dial 911 or proceed to the nearest emergency department, followed by internal notification of the site clinic manager. Staff must still not provide medical advice or symptom interpretation.

## 6. COMPLIANCE AND AUDITING

6.1 All inbound phone calls across the 14 Meridian Health Partners sites are recorded and subjected to monthly quality assurance audits by the Compliance Department. Automated system chat logs are audited weekly.

6.2 Any non-clinical staff member found to have provided clinical advice, interpreted test results, or answered a "should I" question in violation of this standard shall be subject to progressive discipline up to and including immediate termination of employment:
* *First Offense:* Mandatory retraining on MHP-POL-2024-041 and written warning.
* *Second Offense:* Three-day suspension without pay and final written warning.
* *Third Offense:* Termination of employment.

6.3 Automated systems exhibiting drift or failure to trigger the deflection script on Categories 1 through 5 will be taken offline within 2 hours of detection by the IT Systems Administrator until patched and re-certified by the Operations Policy Lead.